

Name : SHIVANYA
Lab No. : 433892468
Ref By : SELF
Collected : 23/7/2023 11:55:00AM
A/c Status : P
Collected at : ZIRAKPUR CC5

Age : 1 Year
Gender : Female
Reported : 23/7/2023 5:01:48PM
Report Status : Final
Processed at : LPL-CHANDIGARH LAB
Plot No. 17, Ground Floor, Industrial Area,
Phase-1, Backside Citroen Showroom,
Chandigarh-160002.

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (Photometry, electrical impedance, vcs technology, calculated & Photometrical capillary)			
Hemoglobin	7.40	g/dL	11.10 - 14.10
Packed Cell Volume (PCV)	25.40	%	30.00 - 38.00
RBC Count	4.89	mill/mm3	3.90 - 5.10
MCV	52.10	fL	72.00 - 84.00
MCH	15.10	pg	25.00 - 29.00
MCHC	29.00	g/dL	32.00 - 36.00
Red Cell Distribution Width (RDW)	21.10	%	12.3 - 17.0
Total Leukocyte Count (TLC)	6.40	thou/mm3	6.00 - 16.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	45.70	%	14.00 - 55.00
Lymphocytes	43.50	%	37.00 - 79.00
Monocytes	10.00	%	2.00 - 12.00
Eosinophils	0.40	%	0.00 - 6.00
Basophils	0.40	%	0.00 - 1.00
Absolute Leucocyte Count			
Neutrophils	2.92	thou/mm3	1.00 - 7.00
Lymphocytes	2.78	thou/mm3	3.50 - 11.00
Monocytes	0.64	thou/mm3	0.20 - 1.00
Eosinophils	0.03	thou/mm3	0.10 - 1.00
Basophils	0.03	thou/mm3	0.02 - 0.10
Platelet Count	452	thou/mm3	200.00 - 550.00
Mean Platelet Volume	8.5	fL	6.5 - 12.0
E.S.R.	10	mm/hr	0.00 - 20.00



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Advised: Hb HPLC to rule out Thalassemia Minor
Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood



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THYROID PROFILE,TOTAL, SERUM (CLIA)			
T3, Total	1.15	ng/mL	1.05 - 2.69
T4, Total	7.00	µg/dL	6.00 - 13.20
TSH	0.74	µIU/mL	0.70 - 6.40

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
2. Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
3. Unbound fraction (Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
4. Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals



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Test Name	Results	Units	Bio. Ref. Interval
AST (SGOT), SERUM (IFCC,without P5P)	42.0	U/L	9.00 - 80.00
ALT (SGPT), SERUM (IFCC,without P5P)	29.0	U/L	7.00 - 35.00
C-REACTIVE PROTEIN; CRP, SERUM (Immunoturbidimetry)	0.70	mg/L	<5.00

Comments

CRP is an acute phase reactant which is used in inflammatory disorders for monitoring course and effect of therapy. It is most useful as an indicator of activity in Rheumatoid arthritis, Rheumatic fever, tissue injury or necrosis and infections. As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, the intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia etc.

WIDAL TEST, SERUM (SLIDE AGGLUTINATION)

Salmonella typhi O (TO)	Non Reactive
Salmonella typhi H (TH)	Non Reactive
Salmonella paratyphi A, H (AH)	Non Reactive
Salmonella paratyphi B, H (BH)	Non Reactive

Interpretation

RESULT	REMARKS
Reactive	Indicates presence of IgM & IgG antibodies against Salmonella spp.
Non-Reactive	Indicates absence of IgM & IgG antibodies against Salmonella spp.

Note:

1. Titres $\geq 1:80$ of "O" antigen & $\geq 1:160$ of "H" antigen for Salmonella typhi and titres $\geq 1:80$ of "H" antigen for Salmonella paratyphi A & B are significant.
2. Rising titres in paired samples taken 7-10 days apart are more significant than a single test.
3. Reactive results indicates ongoing or recent infection by Salmonella spp. and the diagnosis should be confirmed by gold standard test such as Blood culture prior to start of antibiotics.
4. The reactivity will vary with stage of the disease with appearance in 1st week to increase in titres till end



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of 4th week post which it starts decreasing.

- In TAB vaccinated patients, high titres of H antibody of $\geq 1:160$ to each of Salmonellae is observed. They tend to persist for many months and even years while O antibody shows lower titres and disappears within 6 months.
- Antibiotic treatment during 1st week before the appearance of antibodies tend to suppress the immune response in the form of no or decreasing antibody levels.
- False positive results/anamnestic response may be seen in patients with past enteric infection during unrelated fevers like Malaria, Influenzae etc. in the form of transient rise in H antibody in Widal test.
- False negative results may be due to processing of sample collected early in the course of disease (1st week) and immunosuppression.
- Test conducted on serum.

Uses

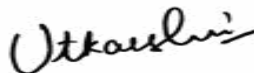
- To diagnose infection due to Salmonella spp. (Enteric fever).
- To monitor the progression of disease.
- To assess the response to therapy (decreasing titres) in patients being treated for Enteric fever.



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-----End of report-----



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IMPORTANT INSTRUCTIONS •Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory . •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician . •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner /Doctor. •The report does not need physical signature. (#) Sample drawn from outside source. If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action. Tel: +91-11-49885050, Fax: - +91-11-2788-2134, E-mail: lalpathlabs@lalpathlabs.com			

